

Rendevor

Patient qualifies to order binders

Reason Denied: _____

REFERRAL CHECKLIST

Referral Date: 2/10

ETA: 2/12

Patient Demographics:

Facesheet (With Insurance/Demographics) ✓

Patient Name: Doe, John

DOB: 3/15/1980

SSN: 123-45-6789

Facility: Sunrise Hills

Multi-Facility:

Hospital: General Medical Center

Contact Name/Number: Sarah Johnson 555-123-4567

Clinical Information:

Height: 165.1 cm

Weight: 81.8 kg

Access: AVF ☐ AVG ☐ TDC ☒ **Found in:** in neph note

HGB (7.0 or greater within 7 days) **8.3** Date: 2/10 Comment: _____

HepB Surface Antigen (neg) Date: 2/10 **HepB Surface Antibody** Date: _____ Value: _____

Diagnosis: ESRD ☒ AKI ☐

Renal Consult Note ☒ 2728 ☐

Dialysis Flowsheets: 2/9

Comment: _____

H&P: ☒ **Medication List:** ☒

Insurance Information:

Patient Primary Insurance: UHC TX Medicaid Star Plus

Insurance approved ☐

Additional Comments:

Admitting under LOA until auth is obtained.

Printable Discharge Form

Site Name	Assigned	Last Activity Date	Respond by Date	Response Status
Sunrise Hills Rehab and Care Center	UnAssigned	2/10/2026 9:09 AM (CT)	2/10/2026 11:07 AM (CT)	Waiting for your response

Sender Name	Response Received	Response Reason	Comment
Nikki Kish	2/10/2026 9:09 AM (CT)	Decision Pending Review	Thank you! Sending out review! Nikki Kish 555-204-5103.

Referral Information – Referral # 99999901 – MRN: TESTMRN-T00000001 – JOHN DOE

Sending Organization: National Health Systems

Sending Facility: General Medical Center

Referral ID: 99999901

Referral Type: Skilled Nursing Facility (SNF)

Date First Sent: 2/2/2026 5:47 PM (CT)

Most Recent Revision: 2/10/2026 9:09 AM (CT)

Respond by Date: 2/10/2026 11:07 AM (CT)

Contact Information**Sarah Johnson**

Phone: (210) 555-5526

Patient Information – Referral # 99999901 – MRN: TESTMRN-T00000001 – JOHN DOE**Name:** JOHN DOE**MRN:** TESTMRNT00000001**Date of Birth:** 3/15/1980 (45 years)**Sex at Birth:** Male**Address:** 100 Main St, Anytown, TX 78000**Home:****Marital Status:** Unknown**SSN:****Race:** Other**Race 2:****Ethnicity:** Unknown**Religion:** Unknown**Emergency Contact 1:** NONE PT STATES**Relationship:** Other**Admission Information****Account #:** T000000001**Patient Type:** Inpatient**Arrival Date/Time:** 2/2/2026 6:51 PM (CT)**Projected Discharge Date:** 2/7/2026 11:00 AM (CT)**Patient Class:** Inpatient**Admit Source:** Information not available**Location:** GMC.IMIS / H.716-A**Level of Care:** Semi Private

Diagnosis & Physician Information

Primary Diagnosis: TRACHEOSTOMY BLEEDING

Initial DRG: MS-DRG (207) RESPIRATORY SYSTEM DIAGNOSIS WITH VENTILATOR SUPPORT >96 HOURS

Admitting Physician: Dr. Michael Lee

Referring Physician: SELF REFERRED

Attending Physician: Dr. Robert Smith

PCP: Dr. William Carter

Financial Information

Financial Class: Managed Care Medicaid

Group Number: TXSTPL

Plan Provider Contact: (888) 702-2202

DOB: 3/15/1980

Employer: UNEMPLOYED

Guarantor Address: 100 Main St, Anytown TX 78000

Plan Number: 000000000

Plan Description: UHC TX MEDICAID STAR PLUS

Insured: JOHN DOE

Pre-Auth/Cert Number: A000000000/I

Guarantor: JOHN DOE

Relationship: Self

Primary Language: English

Allergies: diphenhydramine

GENERAL MEDICAL CENTER AND CHILDREN'S HOSPITAL

Hospitalist History & Physical

REPORT#: 0131-TEST DATE: 01/31/26 STATUS: Signed

PATIENT: DOE, JOHN**UNIT #:** T00000001**DOB:** 03/15/80**AGE:** 45 SEb> M**ACCOUNT#:** T000000001**ROOM/BED:** H.ERTEL-A09**ATTEND:** Dr. Michael Lee MD**History of Present Illness****Chief Complaint:** Shortness of breath**PCP:** Dr. William Carter MD

HPI: This is a 45-year-old male with past medical history significant for ESRD on HD, T2DM, chronic respiratory failure status post tracheostomy on home oxygen 6 L via trach collar, PEG tube in situ, hypertension, hyperlipidemia, cardiac arrhythmia, who presents to the ER for shortness of breath and bleeding around tracheostomy site. Patient missed his dialysis yesterday. Currently complaining of shortness of breath and nausea. Denies any fever or chills. Nursing home resident.

Past Medical / Surgical History

ESRD on HD, T2DM, chronic respiratory failure s/p tracheostomy on home O2 6L via trach collar, PEG tube in situ, hypertension, hyperlipidemia, cardiac arrhythmia. Tracheostomy November 2024; PEG tube placement.

Family History: Diabetes, Heart disease.**Social History:** Denies EtOH; denies recreational drugs; smoking status unknown.**Allergies:** diphenhydramine (From BENADRYL) – Intermediate, SWELLING 01/31/26**Objective / Vital Signs (01/31)**

Date	Temp	Pulse	Resp	B/P	Pulse Ox
01/31	98.3	77–81	18	169–194/71–96	92–97

Weight: 81.818 kg **BMI:** 30.0**Radiology – Chest 1V 01/31:** IMPRESSION: CHF/volume overload. Signed by Dr. D. Patel MD.**Physical Exam**

General: Awake, oriented x3, no acute distress.

Neck: Tracheostomy in place with some bleeding around.

Lungs: Non-labored breathing, clear to auscultation bilaterally, no rales, rhonchi, or wheezing.

Heart: Regular rhythm and rate, no murmurs, normal S1 S2.

Abdomen: Soft, nontender, active bowel sounds, no masses.

Extremities: No significant abnormalities, DP pulse palpable bilaterally. CRT < 3 sec.

Neuro: Normal affect, cranial nerves intact, no focal deficits. Psychiatric: mood stable.

Assessment & Plan

1. Acute on chronic hypoxemic respiratory failure s/p tracheostomy with concern for bleeding around trach site – admit to telemetry, pulmonary consult, wean O2 as tolerated.
2. Volume overload / ESRD on HD (M/W/F) – missed dialysis yesterday; nephrology consulted (Dr. Tecanhuey), labs pending.
3. Rule out acute on chronic CHF – last echo EF 51%, continue GDMT.
4. Hypertension – continue home medications; IV hydralazine PRN.
5. DM Type II – insulin sliding scale.
6. Anemia of chronic disease – no acute blood loss.

ACP: Discussed advance directives / code status – patient wishes FULL CODE.

Electronically Signed by Dr. Michael Lee MD on 01/31/26 at 2122

GENERAL MEDICAL CENTER AND CHILDREN'S HOSPITAL

Nephrology Progress Note | REPORT#: 0209-TEST | DATE: 02/09/26 | STATUS: Signed

PATIENT: DOE, JOHN**UNIT #:** T00000001**DOB:** 03/15/80 **AGE:** 45 **SEX:** M**ACCOUNT#:** T000000001**ROOM/BED:** H.716-A**ATTEND:** Dr. Robert Smith MD**ADM DT:** 02/02/26**AUTHOR:** Dr. James Patel MD**Subjective****Chief complaint:** Tracheal bleed / ESRD**Patient reports:** No complaints. **Nursing Reports:** No complaints.**Objective – Vital Signs (02/09)**

Multiple readings throughout day with BP ranging 133/77 – 173/79; HR 64–77; SpO2 93–100; O2 delivery via Tracheostomy collar at 8–10 L/min.

Weight: 82.2 kg**Labs – 02/09/26 @ 1248**

Chemistry	Result	Ref	Flag
Sodium	133	136–145 mmol/L	L
Potassium	4.2	3.5–5.1 mmol/L	
Chloride	95	98–107 mmol/L	L
CO2	26	20–31 mmol/L	
Anion Gap	16.2	5.0–15.0 mmol/L	H
BUN	56	9–23 mg/dL	H
Creatinine	6.94	0.70–1.30 mg/dL	H
eGFR	10	>60	L
Glucose	237	70–99 mg/dL	H
Calcium	8.3	8.7–10.4 mg/dL	L
Phosphorus	3.5	2.4–5.1 mg/dL	
Magnesium	2.4	1.6–2.6 mg/dL	

Hematology	Result	Ref	Flag
WBC	6.1	4.5–10.5 K/uL	
RBC	2.28	4.20–5.70 M/uL	L
Hgb	6.9	12.0–16.0 g/dL	L
Hct	22.3	36.0–46.0 %	L
MCV	97.8	81.0–98.0 fl	
PLT	152	150–400 K/uL	
RDW	15.3	11.0–14.5 %	H
Random Vancomycin	21.1	<45.0 mcg/mL	

Assessment & Plan – Nephrology 02/09

ESRD – continue routine HD; suspect DM nephropathy; avoid nephrotoxins; adjust meds for ESRD; HD M/W/F.

Volume overload – tolerated ultrafiltration during HD; treat symptomatic CHF; continue to control with hemodialysis, improving.

HTN – improved, continue current regimen, stable.

Anemia of CKD – Hgb low end; started ESA therapy; start Nephro-Vite; iron/iron saturation low → Venofer x3 doses.

Tracheostomy tube change.

Cefepime dosing adjusted: 2g M/W and 3g Friday with HD.

Will not approve PICC/midline – can be given with HD as outpatient.

Case manager to arrange IV antibiotics delivered to SA Resident/Rehab outpatient HD unit; administer at end of HD treatments.

Electronically Signed by Dr. James Patel MD on 02/09/26 at 1849

Active Medication List (as of 02/09–02/11/26)

Medication	Dose	Route	Frequency
Albuterol Sulfate (PROVENTIL UD)	2.5 MG	NEB	Q8H
Amlodipine Besylate (NORVASC)	5 MG	PO	Daily
Budesonide (PULMICORT RESPULE)	0.25 MG	NEB	BID
Calcium Carbonate (TUMS)	500 MG	PO	Q6H PRN
Cefepime HCl (MAXIPIME)	1,000 MG	IV	Daily @1800 (→ adjust at HD)
Ciprofloxacin HCl (CIPRO)	500 MG	Feed-Tube	Daily @1000
Clonidine HCl (CATAPRES)	0.1 MG	PO	Q4H PRN
Epoetin Alfa-epbx (Retacrit)	10,000 UNIT	SUBQ	M/W/F @2100
Formoterol Fumarate (PERFOROMIST)	20 MCG	NEB	BID
Furosemide (LASIX)	40–80 MG	PO	BID @0900,1700
Heparin Sodium – Porcine (Tunneled – Red/Blue Port)	9,000 / 9,500 UNIT	IV-CAT	After Dialysis PRN
Hydralazine HCl (APRESOLINE)	10 MG	IV	Q6H PRN
Hydrocodone/Acetaminophen (NORCO 5/325)	2 TAB	PO	Q4H PRN
Insulin Lispro (AdmeLOG) – Sliding Scale	Per Scale	SUBQ	AC HS / Q2H PRN
Levetiracetam (KEPPRA)	500 MG	PO	BID
Linezolid (ZYVOX)	600 MG	PO	Q12H
Metoprolol Tartrate (LOPRESSOR)	50 MG	PO	BID
Methocarbamol (ROBAXIN)	1,000 MG	PO	Q8H PRN
Morphine Sulfate 2MG Syringe (DC'd)	2 MG	IV	Q6H PRN
Mupirocin (BACTROBAN)	1 APPLIC	Nasal	BID x10 doses
Ondansetron HCl (ZOFTRAN / ODT)	4 MG	IV/SL	Q4–6H PRN
Sevelamer Carbonate (RENEVELA)	1,600 MG	Feed-Tube	With meals
Sodium Chloride 3% Hypertonic	3 ML	NEB	Q12H
Vancomycin HCl (VANCOCIN)	500 MG in NS 100mL	IV	M/W/F @1800 (pharmacy dosing)
Acetaminophen (TYLENOL)	650 MG	PO	Q6H PRN
Glucagon	1 MG	SUBQ	As Directed (hypoglycemia protocol)
Dextrose 50% / Glucose 15 gm / 30 gm	Per protocol	IV/PO	Hypoglycemia protocol

GENERAL MEDICAL CENTER – Laboratory Results – 02/11/26
Patient: DOE, JOHN | MR#: T00000001 | DOB: 03/15/85 | Age: 45/M
Attending: Dr. Robert Smith MD

Hepatology Studies – Collected 02/10/26

Test	Result	Flag	Reference
AB HEP B SURF	REACTIVE	*	NONREACTIVE
AG HEP B SURF	NON REACTIVE		NONREACTIVE
AB HEP B CORE	NON REACTIVE		NONREACTIVE
AB HEP C	REACTIVE	*	NONREACTIVE
AB HB CORE IGM	NON REACTIVE		NONREACTIVE
HBSAB QUANT	180.64	H	<8.00 mIU/mL
HCV RNA QNT PCR	PENDING		

Note: AB HEP B SURF REACTIVE indicates anti-HBsAb detected at levels consistent with immunity. HBSAB QUANT 180.64 mIU/mL confirms immunity (>12 mIU/mL). AB HEP C REACTIVE – HCV RNA PCR pending.

CBC Without Differential – Collected 02/10/26

Test	Result	Flag	Reference
WBC	4.8		4.5–10.5 K/uL
RBC	2.73	L	4.20–5.70 M/uL
HGB	8.3	L	12.0–16.0 g/dL
HCT	26.8	L	36.0–46.0 %
MCV	98.2	H	81.0–98.0 fl
MCH	30.4		28.0–34.0 pg
MCHC	31.0	L	32.0–36.0 %
RDW	15.2	H	11.0–14.5 %
PLT	165		150–400 K/uL
MPV	10.3		8.5–12.0 fL
NRBC %	0		
NRBC ABSOLUTE	0		0 K/uL

Inpatient Hemodialysis Treatment Records (Summary)

Patient: DOE, JOHN | Acct#: T000000001 | MRN: T00000001

Admit MD: Dr. Michael Lee MD | Nephro MD: Dr. James Patel / Dr. Robert Smith

Session Date	Pre Wt (kg)	Post Wt (kg)	Pre BP	Post BP	Access	Tx Time (hrs)	Notes
02/01/26	81	79.5	96/80	86/60	Permacath (RT IJ)	3.5	MD Notified; education on transport; Notify Nephro
02/05/26	60.8	57.5	98/77 P:77	98/82	Permacath	3.5 hrs	Vitals stable, labs reviewed
02/09/26 (2/10 fax)	72.2	79.1	GAS P:64	96/P	TDC (SC)	4 hrs	Heparin 5672 units; RMVD 57L; TMP 36s

Addendum – 02/12/26 by Dr. James Patel MD

Patient has a permacath – RT IJ being used for HD.

Hospitalist Progress Note Attestation – 02/08/26

Patient afebrile with stable respiratory status; improved hemoptysis. Plan for return to The Lev SNF where he dialyzes; pending FiO2 and O2 weaning to SNF parameters.

Cefepime course until 2/17: Cefepime 2g Q Monday and Wednesday after HD; Cefepime 3g Q Friday after HD.

Electronically Signed by Jennifer Brown NP on 02/08/26 at 0955

Electronically Signed by Dr. Carlos Chavez Jr MD on 02/08/26 at 1422

De-identification Notice

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